SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Lapatinib + capecitabine

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive advanced breast cancer · previously treated · NCCP 00217 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control the cancer and symptoms, with repeated review of benefit and tolerance.

Schedule
Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Lapatinib
ORAL HER2 / EGFR KINASE INHIBITOR

- ☐ Diarrhoea and dehydration
- ☐ Rash
- ☐ Liver injury
- ☐ Heart pumping-function or rhythm changes
- ☐ Rare lung inflammation

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea / dehydration
- ☐ Hand-foot syndrome
- ☐ Mouth ulcers / reduced intake
- ☐ Low blood counts and infection
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Renal impairment and medicine interactions

Discuss the treatment aim, alternatives, expected benefit, common and serious effects, fertility and pregnancy precautions, route, monitoring, practical support and the agreed contact plan. The treating team completes the approved individual consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00217 v7.

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